

PRINTABLE HOSPITAL-FAMILY REFERENCE

The Hospital Discharge & Medicare Quick Guide

A plain-English printable reference for families before discharge, rehab, home support, long-term care, or a confusing bill.
Written from a healthcare-worker perspective by Andrew Ciccarelli, RN, BSN. Educational only. Verify with official sources, plan documents, agency notices, SHIP, or a qualified professional.

1 Status	2 Payer	3 Notice	4 Deadline	5 Next call
1 STATUS What is the official status?	2 PAYER Which coverage path applies?	3 NOTICE What written document explains it?	4 DEADLINE What changes today or this week?	5 NEXT CALL Who can verify in writing?

1 **CORE IDEA**
Use this guide to slow the decision down, identify the payer, ask for the written rule, and avoid signing, leaving, or paying before the discharge story is clear.

CHK Who this is for	CHK FAMILY AT DISCHARGE — You are being told home, rehab, SNF, home health, equipment, or private pay may be next.	CHK CAREGIVER ON THE PHONE — You need short questions, not a Medicare textbook.
BILL PATIENT WITH A BILL — You have a hospital, facility, doctor, agency, ambulance, or supplier bill that does not match what you expected.	CHK HEALTHCARE WORKER HELPING FAMILY — You know the care plan, but the payment and paperwork path still feels unclear.	
CHK First four checks	STAT STATUS — Is the patient inpatient, observation, outpatient, discharged, or already transferred?	BILL PAYER — Is the issue Original Medicare, Medicare Advantage, Medicaid, employer insurance, supplemental coverage, or private pay?
DOC DOCUMENT — What notice, order, authorization, MSN, EOB, bill, or denial explains the answer?	CHK DEADLINE — Is there an appeal, discharge, authorization, or payment deadline that needs action today?	
CHK What this means in real life	CHK DO NOT RELY ON HALLWAY ANSWERS — Ask who can verify the answer in writing.	BILL DO NOT TREAT “MEDICALLY NEEDED” AS THE SAME AS PAYER APPROVAL — A care need and a payment rule are separate questions.
DOC DO NOT IGNORE NOTICES — Discharge, denial, and coverage-ending notices can start short response windows.	BILL DO NOT PAY A CONFUSING BILL FIRST — Match it to the processed claim paperwork before paying.	

SOURCE NOTE
Built from Medicare.gov, Medicaid.gov, CMS discharge/appeal resources, SHIP resources, and the official source map at the end of this guide.

AVOID COSTLY ASSUMPTIONS

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The 5 Misunderstandings

Use this page to ask better questions before signing, paying, enrolling, or appealing.

- 1 Separate the issues
- 2 Ask for proof
- 3 Confirm deadlines

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CORE IDEA
Most discharge confusion comes from mixing up care need, hospital status, authorization, program rules, and the final bill. Treat each one as a separate checkpoint.

CHK Five things to separate

- BILL

NEED VS PAYMENT — A patient may need help, but the payer still applies rules, documents, networks, medical-necessity standards, and timing.
- STAT

HOSPITAL BED VS HOSPITAL STATUS — Sleeping in the hospital does not by itself prove inpatient status.
- AUTH

RECOMMENDATION VS AUTHORIZATION — A doctor, therapist, or case manager recommendation is not the same as a plan authorization.
- MCD

MEDICARE VS MEDICAID — Medicare is usually the health-insurance question; Medicaid may become the long-term services/supports or low-income assistance question.
- BILL

PROCESSED CLAIM VS BILL — An MSN or EOB tells the claim story; a bill asks for payment. The numbers should be compared before you pay.

ASK Ask today

- CHK

RULE — “What rule, notice, or plan document are you relying on?”
- CHK

PROOF — “Can you send or print the document that shows this?”
- CHK

TIMING — “What deadline applies if we disagree or need review?”
- CHK

BACKUP — “What happens if authorization is delayed, denied, or ends before the patient is safe?”

CHK What this means in real life

- CHK

BEFORE TRANSFER — Ask whether the receiving facility, agency, or supplier is approved, participating, or in-network for the payer.
- BILL

BEFORE AGREEING TO PRIVATE PAY — Ask what changed, what was denied or not authorized, and what written appeal/review path exists.
- HOME

BEFORE DISCHARGE HOME — Ask what skilled services, equipment, medications, and follow-up appointments are actually ordered.

SOURCE NOTE

Built from Medicare.gov coverage/cost materials, CMS observation-status materials, Medicaid.gov eligibility/LTSS materials, SHIP resources, and Medicare appeal resources.

STATUS CHECK

H

Inpatient vs Observation

Use this page to ask better questions before signing, paying, enrolling, or appealing.

1 Ask current status

2 Confirm start time

3 Save notices

CORE IDEA

H

Hospital status can affect billing and some downstream care rules. Ask for the official status and the date/time it started instead of relying on the room, length of stay, or whether the patient slept overnight.

CHK Know the difference

STAT INPATIENT

— A hospital admission under an inpatient order; ask when the inpatient order began.

CHK OUTPATIENT

— Services may happen inside the hospital without an inpatient admission.

REH SNF QUESTION

— Under Original Medicare, skilled nursing facility rules can depend on a qualifying inpatient hospital stay, with details and exceptions to verify.

CHK OBSERVATION

— An outpatient hospital service, even when the patient stays overnight in a hospital bed.

DOC NOTICE

— Medicare rules include observation-status notice requirements for certain patients.

ASK Ask before discharge

STAT CURRENT STATUS

— “Is the patient inpatient, outpatient, or observation right now?”

DOC NOTICE

— “Was a MOON or other status notice issued, and can we have a copy?”

CHK WHO CAN VERIFY

— “Can utilization review, case management, or billing confirm this in writing?”

CHK TIMING

— “What date and time did inpatient admission begin, if it did?”

REH REHAB IMPACT

— “Could this status affect SNF, rehab, or post-acute coverage rules?”

CHK What this means in real life

CHK EXAMPLE

— A patient can spend multiple nights in the hospital and still have outpatient/observation status for Medicare billing purposes.

CHK IF CONFUSED

— Ask the hospital to explain the status in plain English and identify the appeal or review path if one exists.

CHK PRACTICAL STEP

— Save the status notice, admission order timing, discharge paperwork, and any plan authorization messages together.

SOURCE NOTE

Built from CMS and Medicare.gov materials on inpatient vs outpatient/observation status, MOON notice materials, and Medicare.gov skilled nursing facility materials.

REHAB REVIEW

R

Rehab and Skilled Nursing Facility Care

Use this page to ask better questions before signing, paying, enrolling, or appealing.

1 Document skill

2 Confirm facility

3 Track appeal deadline

CORE IDEA

R

Rehab may be appropriate medically, but payment can depend on the setting, documented skilled need, hospital status, facility participation/network, timing, authorization, and written notices.

CHK

Sort the setting

REH

SNF — Skilled nursing facility care is different from long-term custodial nursing-home care.

STAT

INPATIENT REHAB — A more intensive rehab setting with different criteria and documentation.

HOME

HOME HEALTH — Skilled services at home under specific rules; not the same as round-the-clock home help.

CHK

OUTPATIENT THERAPY — Therapy after discharge without facility admission.

LTC

CUSTODIAL CARE — Ongoing help with bathing, dressing, meals, toileting, supervision, or transportation may follow different rules.

ASK

Ask before transfer

CHK

QUALIFYING STAY — “Does the patient have the hospital status/timing needed for this pathway, if applicable?”

REH

SKILLED NEED — “What skilled nursing or therapy need is documented today?”

CHK

FACILITY FIT — “Is this facility participating, approved, or in-network for this payer?”

AUTH

AUTHORIZATION — “Is authorization approved, pending, denied, or ending?”

AUTH

DISCHARGE PLAN — “What is the expected goal, reassessment date, and backup plan if coverage ends?”

ASK

Call script

CHK

START — “I am trying to verify the rehab payment path before transfer.”

ASK

ASK — “What rule applies, what document proves it, and what can we do if the answer changes?”

CHK

RECORD — Write down the name, department, date, reference number, and next deadline.

SOURCE NOTE

Built from Medicare.gov skilled nursing facility materials, Medicare.gov care comparison materials, Medicare Advantage coverage/appeal materials, CMS appeal resources, and the official source map.

HOME SUPPORT

H

Home Health vs Daily Home Help

Use this page to ask better questions before signing, paying, enrolling, or appealing.

- 1Name the service
- 2Confirm agency
- 3Ask payment path

CORE IDEA

H

Home health, private-duty help, family caregiving, and custodial support are not the same. Ask what service is ordered, who provides it, how often it comes, and who is expected to pay.

CHK Know the difference

HOME

HOME HEALTH — Usually part-time or intermittent skilled care under specific rules, such as nursing or therapy ordered after discharge.

LTC

CUSTODIAL HELP — Bathing, dressing, meals, toileting, supervision, and transportation are daily-living needs, not automatically skilled care.

MCD

MEDICAID/AGING RESOURCES — Long-term home support may involve state Medicaid or local aging resources for eligible people.

CHK

PRIVATE-DUTY AIDE — Help arranged privately or through an agency may be outside Medicare home health rules.

CHK

FAMILY SUPPORT — A discharge plan may assume family availability; ask what happens if family cannot safely provide that level of help.

HOME Ask before discharge home

CHK

ORDER — “What home health, therapy, nursing, equipment, or medication support is actually ordered?”

CHK

FREQUENCY — “How often will visits occur, and what happens between visits?”

CHK

BACKUP — “Who do we call if the patient is not safe at home after discharge?”

CHK

AGENCY — “Which agency accepted the referral, and when is the first visit?”

BILL

PAYMENT — “Who is expected to pay for skilled visits, aides, equipment, supplies, and transportation?”

CHK What this means in real life

CHK

A NURSE VISIT IS NOT 24-HOUR CARE — Home health may be intermittent.

HOME

EQUIPMENT TIMING MATTERS — A walker, oxygen, bed, commode, or supplies may require documentation, supplier rules, delivery timing, or plan authorization.

ASK

AN AIDE IS NOT AUTOMATICALLY INCLUDED — Ask what aide services are approved, ordered, or private pay.

SOURCE NOTE

Built from Medicare.gov home health materials, Medicare.gov durable medical equipment materials, Medicare.gov long-term care materials, Medicaid.gov LTSS materials, and SHIP resources.

EQUIPMENT AND AUTHORIZATION



Equipment, Medications, and Plan Authorization

Use this page to ask better questions before signing, paying, enrolling, or appealing.

- 1 Check order
- 2 Confirm supplier
- 3 Track authorization



CORE IDEA

Equipment, medications, suppliers, and plan authorization can delay a safe discharge. Verify the exact item, supplier, payer rule, authorization status, delivery timing, and backup plan.

CHK Check the item

- CHK

DME — Durable medical equipment can include items such as walkers, wheelchairs, oxygen-related equipment, hospital beds, commodes, or other ordered equipment depending on rules and documentation.
- CHK

ORDER — Ask whether the prescription/order, diagnosis, face-to-face note, therapy note, or medical-necessity documentation is complete.
- CHK

SUBSTITUTION — Ask what safe alternative exists if the exact item is delayed or denied.
- HOME

SUPPLIER — The supplier may need to participate with Original Medicare or be in-network for a plan.
- CHK

DELIVERY — Ask when and where the item will arrive, and whether discharge depends on it.

HOME Ask the plan, supplier, or discharge team

- AUTH

AUTHORIZATION — “Is authorization required, submitted, approved, denied, or pending?”
- CHK

RESPONSIBILITY — “What might the patient owe, and what document explains that?”
- CHK

MEDICATION BRIDGE — “Will the patient have enough medication until follow-up?”
- CHK

NETWORK — “Is this supplier or pharmacy in-network or participating?”
- CHK

APPEAL/REVIEW — “If this is denied or delayed, who starts the appeal or reconsideration?”

CHK What this means in real life

- CHK

DO NOT LEAVE WITHOUT TIMING — If oxygen, mobility equipment, wound supplies, or key medications are needed, ask for the exact handoff plan.
- CHK

SAVE PROOF — Keep the order, delivery confirmation, authorization notice, denial notice, and supplier contact information.
- ASK

CALL EARLY — Equipment and medication delays are easier to solve before discharge than after the patient is already home.

SOURCE NOTE

Built from Medicare.gov durable medical equipment materials, Medicare.gov home health materials, Medicare.gov prescription/plan materials, CMS appeal resources, and Medicare Advantage plan decision materials.

MEDICARE VS MEDICAID

M

Long-Term Care, Custodial Care, and Medicaid

Use this page to ask better questions before signing, paying, enrolling, or appealing.

- 1Sort care type
- 2Check state rules
- 3Ask before private pay

M

CORE IDEA

Do not assume Medicare pays for ongoing custodial long-term care. Medicare and Medicaid answer different questions, and Medicaid long-term services/supports depend on state rules, eligibility, service type, and documentation.

CHKSort the need

- REH

SHORT-TERM SKILLED CARE — Rehab, therapy, skilled nursing, or post-acute care may be reviewed under Medicare or plan rules.
- LTC

LONG-TERM CUSTODIAL CARE — Ongoing help with daily living may not fit Medicare’s medical-care pathway.
- MCD

MEDICAID LTSS — Medicaid may be the relevant program for eligible people who need long-term services and supports.
- CHK

STATE RULES — Income, assets, level-of-care rules, estate recovery, spousal protections, and covered settings can vary by state.
- CHK

DUAL ELIGIBLE — A person can have both Medicare and Medicaid, but the exact help depends on program details.

BILLAsk before agreeing to private pay

- CHK

TYPE OF CARE — “Is this skilled care, custodial care, assisted living, nursing home care, or home/community support?”
- CHK

PROGRAM — “Is Medicare, Medicare Advantage, Medicaid, another payer, or private pay being discussed?”
- CHK

STATE HELP — “Which state Medicaid agency, aging resource, or SHIP counselor should we contact?”
- DOC

NOTICE — “Is there a denial, discharge notice, or coverage-ending notice we need to respond to?”
- AUTH

PLANNING — “Should we speak with an elder-law attorney or qualified Medicaid planning professional before major financial decisions?”

CHKWhat this means in real life

- MCD

MEDICARE VS MEDICAID — Medicare is usually the medical insurance starting point. Medicaid may become the long-term support question for eligible people.
- CHK

TIMING MATTERS — Medicaid applications and long-term care transitions can involve documentation requests and deadlines.
- CHK

DO NOT RELY ON ONE SENTENCE — Ask for the exact program name, agency, notice, and contact number.

SOURCE NOTE

Built from Medicare.gov long-term care materials, Medicaid.gov eligibility materials, Medicaid.gov LTSS materials, Medicaid.gov estate recovery/spousal impoverishment materials, CMS Medicare-Medicaid coordination resources, and SHIP resources.

B

Bills, EOBs, and Medicare Summary Notices

Use this page to ask better questions before signing, paying, enrolling, or appealing.

1 Match documents

2 Ask why owed

3 Correct or appeal

B

CORE IDEA

Do not pay a confusing bill until you match it against the processed claim story: provider bill, MSN, EOB, denial/appeal notice, adjustment, and any financial-assistance option.

DOC

Match the documents

BILL

PROVIDER BILL — The amount a hospital, doctor, facility, agency, ambulance company, or supplier says is owed.

DOC

MSN — Medicare Summary Notice for Original Medicare claims. It explains processed claims and is not a bill.

DOC

EOB — Explanation of Benefits from a private insurer or Medicare Advantage plan. It usually explains processing and is not itself a bill.

CHK

PATIENT RESPONSIBILITY — The amount left after deductible, copay, coinsurance, noncovered service, denial, adjustment, or plan processing.

CHK

FINANCIAL ASSISTANCE — Hospitals may have financial-assistance or charity-care processes; ask before paying if the bill is difficult.

BILL

Four-step bill check

CHK

MATCH — Date of service, provider, location, account number, and service description.

CHK

COMPARE — Bill amount vs MSN/EOB patient responsibility.

ASK

QUESTION — Ask why the patient owes the amount and whether coding, network, authorization, or coordination changed the result.

CHK

ESCALATE — Ask about correction, appeal, rebilling, payment hold, itemized bill, financial assistance, or supervisor review.

ASK

Call script

BILL

BILLING OFFICE — “I am not refusing to pay. I am trying to match this bill to the processed claim before payment.”

AUTH

PLAN/MEDICARE QUESTION — “Can you explain why this amount is patient responsibility and what document shows that?”

CHK

FINANCIAL ASSISTANCE — “Can you screen this account for financial assistance or charity care before we set up payment?”

SOURCE NOTE

Built from Medicare.gov Medicare Summary Notice materials, Medicare.gov appeals materials, Medicare.gov cost materials, HealthCare.gov glossary definitions, and the official source map.

PLAN DECISIONS

A

Medicare Advantage, Prior Authorization, and Appeals

Use this page to ask better questions before signing, paying, enrolling, or appealing.

- 1Check network
- 2Track authorization
- 3Get appeal path

CORE IDEA

A

Medicare Advantage is still Medicare, but plans can use networks, prior authorization, plan rules, notices, and appeal steps. Keep the tone neutral and ask for the written decision path.

- AUTH

Ask the plan or facility

CHK

NETWORK — “Is this hospital, rehab facility, home health agency, supplier, or clinician in-network for this plan?”

DOC

NOTICE — “If the plan denied or ended something, what written notice explains the reason and deadline?”

CHK

APPEAL — “Who files the appeal, what is the deadline, and can it be expedited if the patient may be unsafe?”
- AUTH

AUTHORIZATION — “Was prior authorization required, submitted, approved, denied, or still pending?”
- CHK

ALTERNATIVE — “If this setting or supplier is not approved, what in-network or approved alternative is being offered?”

- CHK

What this means in real life

AUTH

NOT ANTI-PLAN — The goal is not to assume the plan is wrong; the goal is to understand the plan rule, decision, deadline, and next step.

CHK

WRITE IT DOWN — Record the representative name, call reference number, decision date, deadline, and fax/upload instructions.
- CHK

GET SPECIFICS — “Pending authorization” is different from “denied,” “not medically necessary,” “out-of-network,” or “not a plan benefit.”

- CHK

Quick appeal/review checklist

1

DOCUMENT — denial, noncoverage, discharge, or coverage-ending notice.

2

CLINICAL SUPPORT — therapy note, physician order, nursing note, wound/oxygen/mobility note, or unsafe-discharge concern.

3

DEADLINE — exact date/time and whether expedited review is available.

4

DESTINATION — where to send the appeal and how to confirm receipt.

SOURCE NOTE
Built from Medicare.gov Medicare Advantage materials, Medicare.gov appeals materials, CMS managed-care appeal resources, plan decision resources, and SHIP resources.

CALL SCRIPTS

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Scripts and Final Checklist

Use this page to ask better questions before signing, paying, enrolling, or appealing.

- 1 Use short questions
- 2 Write it down
- 3 Escalate carefully

CORE IDEA

?

When overwhelmed, use short questions, ask for the written source, and write down the name, department, date, reference number, document, and deadline.

ASK Call scripts	
STAT HOSPITAL STATUS — “Can you tell me the patient’s official hospital status today and when that status began?”	AUTH DISCHARGE PLAN — “What services, equipment, medications, and follow-up are ordered before discharge?”
REH REHAB/SNF — “What skilled need is documented, and what rule or authorization supports the next setting?”	HOME HOME HEALTH — “Which agency accepted the referral, when is the first visit, and what happens between visits?”
HOME EQUIPMENT — “Is the supplier participating or in-network, and when will the item be delivered?”	AUTH PLAN DECISION — “Is this approved, denied, pending, or ending, and what written notice explains it?”
BILL BILL — “Does this bill match the MSN, EOB, or processed claim record?”	MCD MEDICAID/LTSS — “Which state agency or local aging resource should we contact for long-term support questions?”

CHK Print-and-carry checklist	
1 PATIENT IDENTIFIERS — name, date of birth, Medicare/plan/member ID, Medicaid case number if applicable.	2 CONTACTS — case manager, social worker, plan number, billing office, facility, agency, supplier, SHIP/state resource.
3 DOCUMENTS — discharge paperwork, orders, medication list, authorization notice, denial notice, MSN, EOB, itemized bill.	4 DATES — admission/status date, discharge date, service date, appeal deadline, authorization deadline, first home visit.
5 OPEN QUESTIONS — what is approved, what is pending, what might be owed, who verifies, and what happens if the plan fails.	

BILL Last rule before signing or paying	
CHK PAUSE — “I need to verify the payer, document, and deadline before I agree.”	CHK ESCALATE — Ask for a supervisor, case manager, billing review, plan appeal team, SHIP counselor, state Medicaid agency, or qualified professional.
CHK RECORD — Keep a simple call log with name, department, date, reference number, and exact next step.	CHK UPDATE — If the answer changes, ask for the updated notice or document.

SOURCE NOTE

Built from the official source map below, Medicare.gov appeals materials, CMS plan decision resources, SHIP locator resources, Medicaid.gov resources, and guide-hub tool references.

OPTIONAL TRUST LAYER



Endnotes and Source Map

Official sources used to keep the short guide grounded without turning it into a textbook.

1 Medicare.gov — Skilled nursing facility care: https://www.medicare.gov/coverage/skilled-nursing-facility-care	2 Medicare.gov — Inpatient or outpatient hospital status: https://www.medicare.gov/basics/your-medicare-rights/your-protections/your-rights-in-the-hospital	3 CMS — Medicare Outpatient Observation Notice: https://www.cms.gov/medicare/forms-notices/beneficiary-notices-initiative/moon
4 Medicare.gov — Home health services: https://www.medicare.gov/coverage/home-health-services	5 Medicare.gov — Durable medical equipment: https://www.medicare.gov/coverage/durable-medical-equipment-dme-coverage	6 Medicare.gov — Long-term care: https://www.medicare.gov/coverage/long-term-care
7 Medicare.gov — Medicare Advantage Plans: https://www.medicare.gov/health-drug-plans/health-plans/your-health-plan-options	8 Medicare.gov — Original Medicare: https://www.medicare.gov/basics/get-started-with-medicare/get-more-coverage/your-coverage-options/compare-original-medicare-medicare-advantage	9 Medicare.gov — Medigap: https://www.medicare.gov/health-drug-plans/medigap/basics/how-medigap-works
10 Medicare.gov — Appeals: https://www.medicare.gov/claims-appeals/how-do-i-file-an-appeal	11 Medicare.gov — Medicare Summary Notices: https://www.medicare.gov/basics/forms-publications-mailings/mailings/claims-and-appeals/medicare-summary-notice	12 Medicare.gov — Medicare costs: https://www.medicare.gov/basics/costs/medicare-costs
13 Medicaid.gov — Eligibility: https://www.medicaid.gov/medicaid/eligibility/index.html	14 Medicaid.gov — Long-term services and supports: https://www.medicaid.gov/medicaid/long-term-services-supports/index.html	15 Medicaid.gov — Estate recovery: https://www.medicaid.gov/medicaid/eligibility/estate-recovery/index.html
16 Medicaid.gov — Spousal impoverishment: https://www.medicaid.gov/medicaid/eligibility/spousal-impoverishment/index.html	17 CMS — Coordination between Medicare and Medicaid: https://www.cms.gov/medicare/coordination-benefits-recovery/medicare-medicaid-coordination	18 Medicare.gov — Medicare Savings Programs: https://www.medicare.gov/basics/costs/help/medicare-savings-programs
19 Medicare.gov — SHIP help: https://www.shiphelp.org/	20 HealthCare.gov — Health insurance glossary: https://www.healthcare.gov/glossary/	